

Executive Summary

This memo outlines NexusHealth AI's current financial position, our 3-year forward projections, and a detailed breakdown of how we intend to deploy the \$5,000,000 Series A investment to achieve sustainable growth and reach cash flow breakeven by month 18 post-close.

Section 1: Current Financial Position (Pre-Investment Baseline)

As of the date of this memo, our financial position is as follows:

Our cash balance stands at \$1,500,000. Against a net monthly burn of \$400,000, this gives us approximately 3.75 months of runway — a fact that underscores both the urgency of closing this round and our confidence that the pipeline and traction metrics justify it.

Monthly Revenue: \$100,000 (equating to \$1,200,000 ARR across 6 active hospital contracts)

Monthly Expenses: \$500,000, broken down as follows:

Personnel costs represent the largest line item at approximately \$290,000 per month, covering our current team of 18 full-time employees across engineering, product, sales, and customer success. Cloud infrastructure and model compute costs run \$55,000 monthly — a figure that scales modestly with new client deployments due to our multi-tenant architecture. Sales and marketing spend is \$75,000 per month, currently weighted toward conference presence and outbound SDR activity. General and administrative costs, including legal, finance, and office operations, account for the remaining \$80,000 monthly.

Net Monthly Burn: \$500,000 expenses minus \$100,000 revenue equals a net cash burn of \$400,000 per month.

It is worth noting that our gross margin on recognized revenue is approximately 74%, meaning the cash drag is not a product efficiency problem — it is a deliberate investment in growth ahead of a revenue base that is still maturing.

Section 2: Use of \$5,000,000 Series A Proceeds

The \$5,000,000 investment will be deployed over a 24-month period with two primary objectives: accelerating revenue growth through sales expansion, and deepening our product moat through engineering investment. Secondary allocations cover customer success capacity and working capital.

Engineering Hires — \$1,750,000 (35% of total raise)

This is our single largest allocation and reflects a deliberate decision to widen the technical gap between NexusHealth AI and any emerging competition. Over the first 18 months, we will hire 7 additional engineers, including two senior ML engineers focused on model accuracy and generalization across EHR environments, two backend engineers to build new EHR integration connectors beyond our current Epic and Cerner support, one data infrastructure engineer to scale our FHIR pipeline as client volume grows, one product engineer focused on our clinician-facing alert interface, and one security and compliance engineer to support SOC 2 Type II certification and HIPAA audit readiness as we enter enterprise procurement with larger health systems.

At fully-loaded costs averaging \$175,000–\$200,000 per engineer per year, this allocation covers both hiring and 12–18 months of fully-loaded compensation for each new role.

Sales Expansion — \$1,500,000 (30% of total raise)

Our current sales team of three reps has produced \$1.2M in ARR. The math on scaling is straightforward: each additional quota-carrying rep, at a target of \$600,000–\$800,000 in new ARR annually, pays back their cost within 12 months at our current contract values.

This allocation funds the hiring of 6 additional quota-carrying account executives with existing health system relationships, one Director of Sales to manage the expanded team, an increase in SDR capacity from 1 to 3 to support pipeline generation, a 3x increase in conference and field marketing spend to drive awareness across the regional health system segment, and investment in a channel partnership program to leverage EHR vendor relationships for co-sell and referral motions.

Customer Success & Implementation — \$750,000 (15% of total raise)

Revenue retention is our most important near-term financial metric. Our current 92% gross retention is strong, but it is maintained by a CS team that is close to capacity. This allocation funds 3 additional implementation and customer success managers, a structured 90-day onboarding playbook with embedded outcome tracking, and a client health scoring system to identify and address expansion and at-risk signals proactively.

General & Administrative / Working Capital — \$500,000 (10% of total raise)

This covers incremental G&A costs associated with operating as a Series A company — specifically audit and accounting infrastructure, D&O insurance, legal fees associated with enterprise contract negotiations, and a modest working capital buffer.

Total Allocated: \$4,500,000. The remaining \$500,000 is held as a contingency reserve against delayed sales cycles or higher-than-projected ramp costs in the new engineering hires.

Section 3: 3-Year Financial Projections

The projections below are built on bottom-up assumptions tied to sales headcount, average contract values, and historical sales cycle lengths of 3–5 months for mid-size health systems.

Year 1 (Months 1–12 Post-Investment Close)

In the first year, sales ramp takes 3–4 months as new account executives build pipeline. We project closing 12–15 new hospital contracts at an average ACV of \$200,000, bringing total ARR to approximately \$4,800,000 by month 12. Monthly revenue exits the year at approximately \$400,000. Total expenses will increase as new hires come online, peaking at approximately \$750,000 per month in months 8–10 before new revenue begins to offset the cost base meaningfully. Net cash burn for Year 1 is projected at approximately \$3,200,000.

Year 2 (Months 13–24)

By the start of Year 2, our sales team is fully ramped and our pipeline is substantially larger. We project closing an additional 20–25 contracts, including our first multi-facility health system deals at ACVs of \$350,000–\$500,000. We project exiting Year 2 with \$10,500,000 in ARR and monthly revenue of approximately \$875,000. Critically, we project reaching cash flow breakeven in month 18, as monthly revenue crosses the monthly expense run rate. Total expenses by end of Year 2 are projected at approximately \$850,000 monthly, held in check by the leverage inherent in our SaaS model — each new contract adds minimal marginal cost. Net cash position at end of Year 2, assuming \$5M investment and Year 1 burn, is projected at approximately \$1,100,000–\$1,400,000, providing a thin but sufficient runway into a Series B raise from a position of demonstrated efficiency.

Year 3 (Months 25–36)

Year 3 is a profitability and Series B setup story. With a fully ramped team, mature channel partnerships, and strong renewal and expansion revenue from our growing installed base, we project exiting Year 3 with \$18,000,000 in ARR. Gross margins improve to approximately 77% as infrastructure costs scale sublinearly with revenue. We project net income turning positive in Q3 of Year 3 on a monthly basis. This positions NexusHealth AI to either raise a Series B from a place of leverage — not urgency — or to evaluate a path to profitability on existing capital.

Section 4: Key Financial Risks & Mitigations

No financial projection is complete without an honest accounting of the assumptions that could prove wrong.

The primary risk is sales cycle elongation. Health system procurement can extend beyond our modeled 3–5 months, particularly in a macro environment where hospital CFOs face margin pressure. Our mitigation is a pipeline coverage ratio target of 4x quota and an active LOI-stage pipeline of \$2.1M already in place at the time of this memo.

The second risk is engineering talent cost inflation. We have modeled conservatively at \$185,000 fully-loaded per engineer, but the ML talent market remains competitive. We have partially mitigated this through early conversations with three senior candidates already at offer stage.

Finally, EHR integration complexity can extend deployment timelines and delay revenue recognition. Our investment in a dedicated security and infrastructure engineer, and our expanding pre-built connector library, is designed to bring average deployment time below 6 weeks consistently.

Conclusion

NexusHealth AI is at an inflection point. Our unit economics are sound, our retention is strong, and our pipeline validates that product-market fit is real. The \$5,000,000 Series A investment, deployed as described above, gives us a clear and credible path to \$18M ARR, cash flow breakeven at month 18, and a Series B raise from a position of strength.

We are raising this round with urgency and conviction.

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This document is confidential and intended solely for prospective investors under NDA. Forward-looking statements are based on management estimates and are subject to material risks and uncertainties.